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CLINICAL EVENTS COMMITTEE CHARTER

KISS

Keep bIfurcation Single stenting Simple

Sponsor: CERIC (CEnter for European Research Initiatives in Cardiovascular medicine)

3 rue des Granges – 1204 Geneva – SWITZERLAND

CEC Charter effective date:
October 14, 2021

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My signature below serves to confirm that I have reviewed and approved this Charter, including all Appendices. I hereby confirm I understand my responsibilities as a member of this Clinical Events Committee and I confirm that I do not have any conflict of interest with this research.

DocuSigned by:

Signed: Emanuele Barbato **Date:** 22-ott-21

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Name: Emanuele Barbato, CEC Chairperson

Signed: **Date:** 15/11/2021

Name: Yves Louvard, CEC member

DocuSigned by:

Signed: Dr Fina Mauri **Date:** 25-oct.-21

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Name: Fina Mauri, CEC member

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Approval signature CERC (Cardiovascular European Research Center):

DocuSigned by:

Signed: Marie-Claude MORICE **Date:** 22-oct.-21

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***Marie-Claude Morice*, MD**

Approval Signature [Sponsor]

DocuSigned by:

Signed: _____ **Date:** 25-oct.-21

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***Stéphane Cook*, MD, PhD**

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1 Scope of the document

The present document summarizes all aspects of the Clinical Events Committee (CEC) set up by CERC (Cardiovascular European Research Center) for the KISS Study, of which CERIC (CEnter for European Research Initiatives in Cardiovascular medicine) is the study Sponsor and Medtronic is the Grant Giver. It comprises CEC responsibilities, staffing selection, event review process including identification of endpoints requiring adjudication, the method for tracking source documents and the process for reporting CEC decisions.

2 Definitions

The CEC is an independent group of individuals with pertinent expertise who review and adjudicate important endpoints, which occurred in clinical studies and have been reported by study investigators.

Definitions of clinical endpoints to be adjudicated are established prior to the study start and are detailed in the Clinical Investigation Plan (CIP) of the KISS study for use by the Committee members (see Appendices #1, #2 and #3).

3 Objectives

The objective of the committee is to review clinical events experienced by research subjects who are participating in a clinical study and to adjudicate them as to whether or not they represent safety and efficacy endpoints as defined in the CIP. CEC adjudications may or may not correspond to the conclusions of the investigators who declared the events. Should a discrepancy arise, the committee's conclusion will serve as the final decision for the purpose of result publication.

The importance of the CEC is to ensure that all key events that have been reported are judged uniformly by a single independent group using the uniform definitions as stated in the CIP and in this charter.

4 CEC set-up

4.1 Composition of a CEC

The committee is managed by CERC and is designated as the Clinical Events Committee for the KISS study.

The CEC is composed of three physicians, including the CEC chairperson. The CEC members are interventional cardiologists. These physicians are not participants in the

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clinical study, have no conflict of interest regarding the study device under evaluation and are independent from the study Sponsor, clinical sites participating in the study, data management and study monitoring.
Each member is responsible for advising the CERC of any changes in the conflict of interests that occur during the course of the study and should resign from the CEC in case of significant change that can affect the trial.

4.2 CEC member selection

The CEC members of the KISS study have been chosen based on their personal expertise in the domain of the trial and as CEC members during their participation in CECs for other studies managed by CERC or have been proposed by the study Principal Investigator or study Sponsor.

The CEC members of the KISS study are:

1. Emanuele Barbato (chairperson)
2. Yves Louvard
3. Fina Mauri

Please refer to Appendix #4 for contact details of the CEC members.

CEC membership is for the duration of the adjudication activities. If a member decides to leave the CEC or can no longer meet the event review and adjudication requirements, he or she must inform CERC in writing. CERC, the CEC chairperson and CERIC will identify a replacement. In addition, if a member is unable to fulfill his/her responsibilities, he/she will be asked to step down and will be replaced.

4.3 CEC Chairperson selection and responsibilities

One of the CEC members is assigned as the CEC Chairperson and is chosen prior to the first CEC meeting. The Chairperson selected for this study is Emanuele Barbato.

The Chairperson's responsibilities are to oversee that the endpoints are adjudicated in accordance with the CIP and this CEC Charter.

4.4 Documents provided to and to be collected from the CEC members

Along with the invitation to participate in the CEC, a confidentiality agreement is sent to the physicians with the request to return a signed confidentiality agreement to CERC.

Then the following documents are sent to the members:

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- Clinical Investigation Plan – Final approved version;
- Instruction for Use of the study device and any other documentation in order to assess the risks and benefits of the investigational product;
- with the request for signature and return of the signed document to CERC:
    - CEC Charter,
    - CEC member's financial contract,
    - Conflict of Interested Statement.

In addition to the above listed documents, CEC members are requested to provide their current dated and signed Curriculum Vitae.

During the course of the study, updates to the above documents are provided to CEC members, when applicable.

All relevant documents regarding the CEC are retained and filed in the CEC section of CERC's electronic Trial Master File (eTMF) of the KISS study.

5 CERC CEC Management team and responsibilities

The committee is supported by the CERC CEC management team as non-voting members (two CERC employees involved in the KISS study – usually Clinical Project Leader (CPL) and Clinical Trial Assistant (CTA)).

Its responsibilities are:

- preparation and scheduling of the CEC meetings,
- ensuring the CEC members adhere to all CEC procedures outlined in the CIP and in the CEC charter,
- recording the adjudication information,
- writing of the meeting minutes.

6 Event adjudication form design

The design of the Event Adjudication Form is based on the definition of the primary and secondary endpoints as defined in the CIP (see Appendix #1). It was performed after finalization of CIP and designing of the study case report form (see Appendix #4 for sample Event Adjudication Form).

Decisions and suggestions related to changes in the Event Adjudication Form template may be made at the latest during the first CEC session. The need for modifications will be discussed by the Chairperson, CEC members, the CERC Medical Director and proposed to the Sponsor, and each new version issued is validated by the CEC members and CERC Medical Director by signing a validation form.

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Upon finalization, the Event Adjudication Form is converted to PDF format for purposes of archiving with proper version control and date of finalization. The form is distributed in PDF format to all relevant personnel, including the Study Sponsor. The final form is filed in the CEC section of the eTMF as Appendix #4 to this charter.

The adjudication forms are provided on an electronic platform (electronic adjudication form), which is linked to the electronic Case Report Form (eCRF) of the KISS study.

7 Meeting preparation

To prepare for the CEC meetings, the CPL continuously follows Adverse Event reporting in this study. The full listing of Adverse Events is presented to CERC Medical Director (MD) for identification of events requiring adjudication.

7.1 Events to be reviewed

The CEC review and adjudicate all clinical endpoints of the study reported by the investigational sites of the KISS Study throughout the complete follow up phase (12 months).

When study endpoint related events have occurred, clinical investigational sites are responsible for ensuring the information is entered into the eCRF in a timely manner. The study monitors are responsible for ensuring that endpoint events have been duly monitored prior to adjudication. However, also unmonitored events may be adjudicated if needed.

A cumulative Adverse Event line listing is provided to the CEC by CERC. The CEC may also adjudicate events which are not endpoint related if deemed necessary. At each meeting, 10-20 % of events that have previously classified by the clinical investigational sites and by CERC MD as not end-point related, are reviewed by the CEC for confirmation.

In the case that an additional Adverse Event is identified by any of the involved parties (CEC, Study Sponsor, study monitor) that might fulfill criteria for one of the study endpoints (e.g. elevated cardiac enzymes, chest pain etc.) then these events are also presented for adjudication.

There might be instances in which events that have already been adjudicated may require re-adjudication. These include, but are not limited to the following:

- Events for which additional relevant source documents become available
- Events undergoing quality control check by CERC for obvious errors

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8 CEC meeting procedure

8.1 Events requiring adjudication

All study endpoint-related Adverse Events (refer to Appendices #2 and #3) as listed below will be presented to the CEC members for adjudication:

- Death
- Myocardial Infarction
- Revascularization (Re-PCI and CABG)
- Stent Thrombosis

8.2 Meeting schedule

CEC meetings are scheduled based on the CIP requirements, the estimated number of events, and the trial state of advancement.

Two (2) CEC sessions (1 day meeting at the end of inclusions (primary endpoint) and 1 meeting at 1 year follow-up) are planned for the KISS study.

If additional meetings are required or due to an increased number of events, then agreement between the study Sponsor and CERC should be obtained.

Each CEC meeting should have a minimum of 3 members including the CEC Chairperson present. In case a CEC member cannot attend a scheduled meeting due to an emergency, a CEC meeting may exceptionally take place with only two members, one of whom must be the Chairperson, due to his responsibilities (see section 4.3.).

Once a CEC meeting has been scheduled, the CEC committee members will be provided with meeting dates as well as time and place via email. An adequate amount of time should be scheduled for each meeting, and the members should be made aware of the estimated length of each meeting.

The last adjudications should occur after all patients have completed their follow-up and all CIP required event forms in the eCRF have been monitored by the study monitors and signed off by the site investigators.

Members will be informed of all study timelines that may affect the adjudication schedule such as interim, progress or final reports.

If for any reason a CEC physical meeting cannot be held, the CEC meeting may also be organized as web meeting. Once a CEC web meeting has been scheduled, all CEC committee members will be contacted via email providing web meeting dates and time information. All CEC members will receive access to the CEC source document database prior to the CEC web meeting.

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8.3 CEC training

During the first CEC meeting a Sponsor's representative may participate via phone or in person to provide a study overview of the study to the CEC members. The CPL will present primary and secondary endpoints with their definitions (see Appendices #2 and #3) as well as the adjudication forms and the adjudication process to the CEC.

All additional questions or comments that may arise will be clarified to the CEC members before the initiation of the adjudication process. After the members are fully informed on the details of the study and the adjudication of events, the adjudication process will begin in closed-door session without Sponsor representatives.

The CEC members will be reminded of the study endpoints at the beginning of subsequent meetings. Study endpoint definitions will be provided to assist the CEC members in the accurate adjudication and classification of the events.

8.4 Adjudication process

The events to be adjudicated will be presented to the CEC in tabular form. The CERC CEC management team provides the CEC members with all source documents available regarding the events. Each event is reviewed for its potential to be an endpoint related event. Every endpoint related event will be reviewed, discussed and adjudicated according to study definitions (Appendices #3). When all CEC members agree on the definition of the event, the adjudication form will be completed accordingly.

Decision on adjudication may be reached as follows:

- all CEC members agree on the adjudication
- if not, all CEC members agree on the adjudication, the majority opinion will prevail
- if majority cannot be reached, the Chairperson’s opinion will prevail

In the exceptional case that a CEC meeting is held in the presence of two CEC members only (refer to section 8.2) an event will be stated as adjudicated, if the two attending CEC members agree on the adjudication. Conflicting cases will be discussed again in the presence of all CEC members during the following scheduled plenary meeting.

The adjudication results will be directly entered into the electronic adjudication form. Upon completion of the form and verification by the CEC members and CERC management team, the form will be signed electronically by the CEC Chairperson.

If additional source documentation is requested by the CEC members in the course of the adjudication process, the request is noted in the minutes and is followed up after the meeting. All requests for additional source documents will be communicated to the study

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monitors, who will forward the request to the clinical investigation sites. Once the requested document is available, the event will be re-adjudicated. In case the clinical investigation site is unable to provide additional source documents, the CEC will be requested to adjudicate the event based on the available information.

In case the CEC discovers an event that has not been reported by the sites, this information will be captured in the Adjudication Form and the event will be adjudicated as “added event”.

All members including voting and non-voting in attendance at the meeting will sign the attendance list. The attendance list will be saved in the CEC section of CERC’s eTMF of the KISS study.

8.5 Meeting minutes

After completion of the CEC meeting and adjudication of all the events for that meeting, minutes will be written by a member of the CERC CEC management team and subsequently reviewed and validated by the CEC Chairperson. Any changes or corrections should be made prior to finalizing the minutes and signature by the CEC Chairperson and CPL. The minutes are kept in the CEC section of the eTMF and will be sent to the study Sponsor.

9 Provision for changes to the CEC Charter

The Charter may be amended as needed during the course of the study. All amendments will be documented with sequential version numbers and revision dates, and will be recorded on the Document History Form which will be filed in the CEC section of the eTMF. Such revisions may include but are not limited to:

- changes in CEC membership,
- changes in workflow,
- addition of study documents relevant to the CEC and
- changes to the CIP or other circumstances which may impact CEC adjudication.

Any change in the Charter, any deviation from the accepted CEC processes and any change in CEC membership will be notified to the study Sponsor and duly documented by CERC in the CEC section of the eTMF.

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10 CEC Charter Appendices

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